

Yes, according to the source provided, it is **entirely possible** to create this application by integrating current legislative data with the **NYC Open Data API** 1, 2. The source explicitly outlines a "Digital Diabetes Prevention Framework" designed for this purpose 1. To make the application a reality, the source identifies the following key components for your development:

1. Backend: Data Integration

You would use the **NYC Open Data API** to pull real-time information that identifies community needs 3, 2. Specifically, you would fetch:

- **"Community Health Survey"** datasets to understand regional health trends 3.
- **"Diabetes Prevalence"** datasets to locate areas with the highest concentrations of the disease 3, 4.
- **Demographic Data** to identify where AAPI populations are growing the fastest (noted as over 11% of New York State) to ensure the map overlaps high-need areas with high-population areas 4.

2. Logic: AAPI-Specific Risk Assessment

The core functionality of your application would rely on the **legislative logic** found in proposed health bills 1, 2.

- Instead of standard medical screenings, your app's "Risk Calculator" would apply the **lower Body Mass Index (BMI) thresholds** specifically advocated for the AAPI community to flag health risks earlier than traditional tools 3.

3. Frontend: Multilingual Accessibility and Resources

Because **43% of Asian Americans in NYC** have Limited English Proficiency (LEP), the source specifies that the application must be **multilingual** to be effective 5.

- **Multilingual Navigation:** The tool should provide information in languages like **Bengali, Chinese, Korean, and Urdu** so users do not have to rely on children to translate complex medical data 5.
- **Provider Directory:** You would map the **150+ non-profits** funded by the **\$54.35 million AAPI Equity Budget**, such as the South Asian Council for Social Services (SACSS), which provides support in over 20 languages 4, 6, 2.
- **Compliance Tracking:** The app can highlight which hospitals are compliant with the **Hospital Language Access Law (S6288B/A387B)**, ensuring users find facilities with skilled interpreters and dedicated coordinators 6.

By merging these specific datasets and legislative mandates, the source concludes that your application would function as a vital tool for **health equity**, allowing vulnerable populations to access life-saving screenings in their own language 2.